

# Susceptible host dynamics explain pathogen resilience to perturbations

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## Abstract

Interventions to slow the spread of SARS-CoV-2 significantly disrupted the transmission of other pathogens. As interventions lifted, whether and when human pathogens would eventually return to their pre-pandemic dynamics remains to be answered. Here, we present a framework for estimating pathogen resilience based on how fast epidemic patterns return to their pre-pandemic dynamics. By analyzing time series data from Hong Kong, Canada, Korea, and the US, we quantify the resilience of common respiratory pathogens and further predict when each pathogen will eventually return to its pre-pandemic dynamics. Our predictions are able to distinguish which pathogens should have returned already, and deviations from these predictions reveal long-term impacts of pandemic perturbations. We find a faster rate of susceptible replenishment underlies pathogen resilience and sensitivity to both large and small perturbations. Overall, our analysis highlights the persistent nature of common respiratory pathogens compared to vaccine-preventable infections, such as measles.

## Significance Statement

COVID-19 interventions slowed the transmission of many respiratory pathogens in different ways, raising questions about the mechanisms driving the variation in responses to interventions. To address this gap, we characterized the sensitivity of pathogen transmission to perturbations by quantifying how fast each pathogen returned to its pre-pandemic circulation patterns. We analyzed data from Hong Kong, Canada, Korea, and the US, and showed that common respiratory pathogens are far

37 less sensitive to perturbations than measles, a vaccine-preventable infection. Finally,  
38 we showed that the speed of replenishment of the susceptible population—for exam-  
39 ple, through waning immunity—largely determines this sensitivity, suggesting that  
40 the persistence of common respiratory pathogens is likely driven by rapid susceptible  
41 replenishment.

## Introduction

Non-pharmaceutical interventions to slow the spread of SARS-CoV-2 disrupted the transmission of other human respiratory pathogens, adding uncertainties to their future epidemic dynamics and their public health burden [1]. As interventions lifted, large differences in outbreak dynamics were observed across different pathogens in different countries, with some pathogens exhibiting earlier and faster resurgences than others [2, 3, 4]. These differences likely reflect variation in interventions, pathogen characteristics, immigration/importation from other countries, and pre-pandemic pathogen dynamics [5]. Therefore, comparing the differential impacts of pandemic perturbations can provide unique opportunities to learn about underlying pathogen characteristics in different populations, such as pathogens' transmissibility and duration of immunity [6].

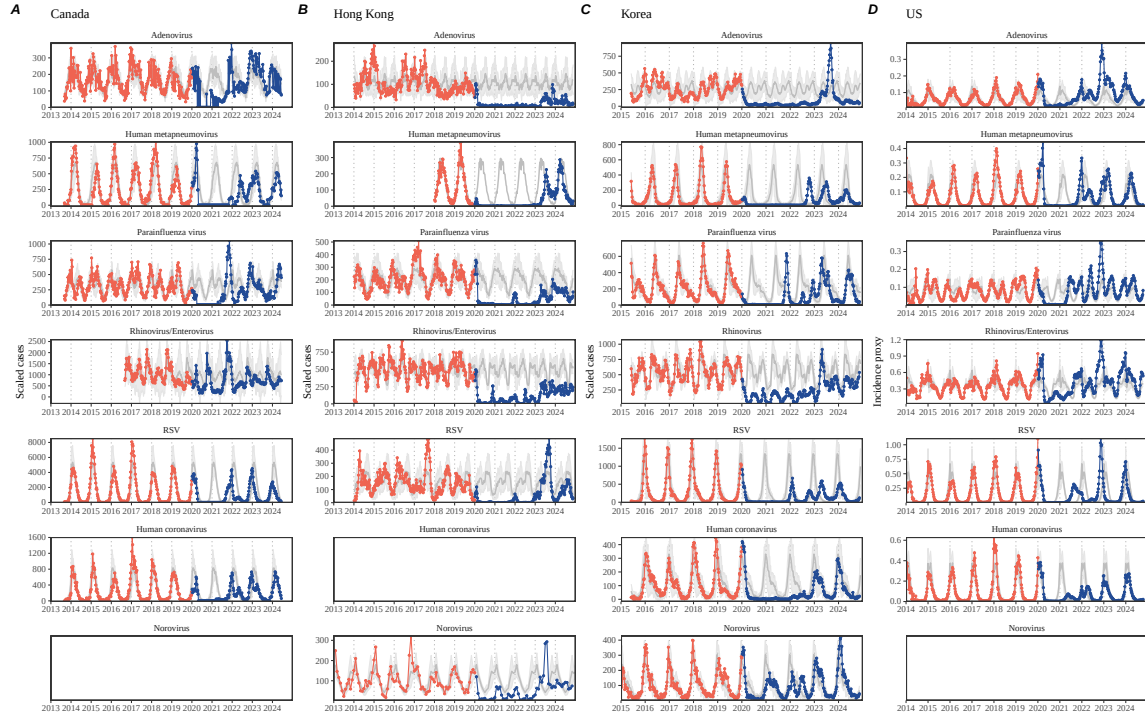


Figure 1: **Observed heterogeneity in responses to pandemic perturbations across respiratory pathogens and norovirus in (A) Canada, (B) Hong Kong, (C) Korea, and (D) US.** Red points and lines represent data before 2020. Blue points and lines represent data since 2020. Gray lines and shaded regions represent the mean seasonal patterns and corresponding 95% confidence intervals around the mean. Mean seasonal patterns were calculated by aggregating cases before 2020 into 52 weekly bins and taking the average in each week. Cases were scaled to account for changes in testing patterns (Materials and Methods).

Even though more than five years have passed since the emergence of SARS-CoV-

2, we still observe persistent changes in pathogen dynamics following the pandemic perturbations. For example, compared to pre-pandemic, seasonal patterns, human metapneumovirus in Korea seems to circulate at lower levels, whereas RSV in Korea seems to exhibit different seasonality (Figure 1). These observations suggest the possibility of a long-term change in pathogen dynamics following the pandemic perturbations, which might be driven by a long-term shift in human behavior or population-level immunity [7, 8]. For example, the emergence of SARS-CoV-2 could have caused a long-term shift in population-level immunity through interactions with other pathogens [9], especially seasonal coronaviruses [7, 10, 11]. The possibility of a long-lasting impact of the pandemic perturbations poses an important question about future outbreaks: can we predict whether and when other pathogens will eventually return to their pre-pandemic circulation patterns?

So far, most analyses of respiratory pathogens after pandemic perturbations have focused on characterizing the timing of rebound (i.e., when the incidence first returns above historical thresholds) and linking this timing to the relaxation of intervention measures or changes in population susceptibility [1, 12, 5]. While quantifying this timing is useful for public health agencies, epidemic rebound differs from the return to historical dynamics—the rebound marks the initial return of pathogen activity without any information about the subsequent dynamics. Instead, we seek to characterize how fast (and whether) a pathogen returns to its pre-pandemic dynamics by utilizing dynamical information after the perturbation. Measuring this rate of return is useful because it allows us to quantify the ecological resilience of a host-pathogen system, which can inform responses to future interventions [13, 14, 15, 16].

In this study, we explore theoretical and statistical approaches to characterizing the resilience of a host-pathogen system based on how fast the system recovers from perturbation. We begin by laying out representative scenarios that capture the potential impacts of pandemic perturbations on endemic pathogen dynamics and illustrate how resilience can be measured by comparing the pre- and post-pandemic dynamics of susceptible and infected hosts. In practice, information on susceptible hosts is often unavailable, making this theoretical approach infeasible. Instead, we utilize a mathematical technique to reconstruct attractors from the data [17], which allows us to measure the rate at which the host-pathogen system approaches this empirical attractor after a perturbation; we define this rate to be the empirical resilience of the host-pathogen system. We use this method to analyze pathogen surveillance data for respiratory and non-respiratory pathogens from Canada, Hong Kong, Korea, and the US. Finally, we show that susceptible host dynamics explain variation in pathogen resilience and demonstrate that more resilient pathogens will be less sensitive to perturbations caused by demographic stochasticity, providing a direct link between pathogen resilience and persistence.

## Conceptual introduction to pathogen resilience

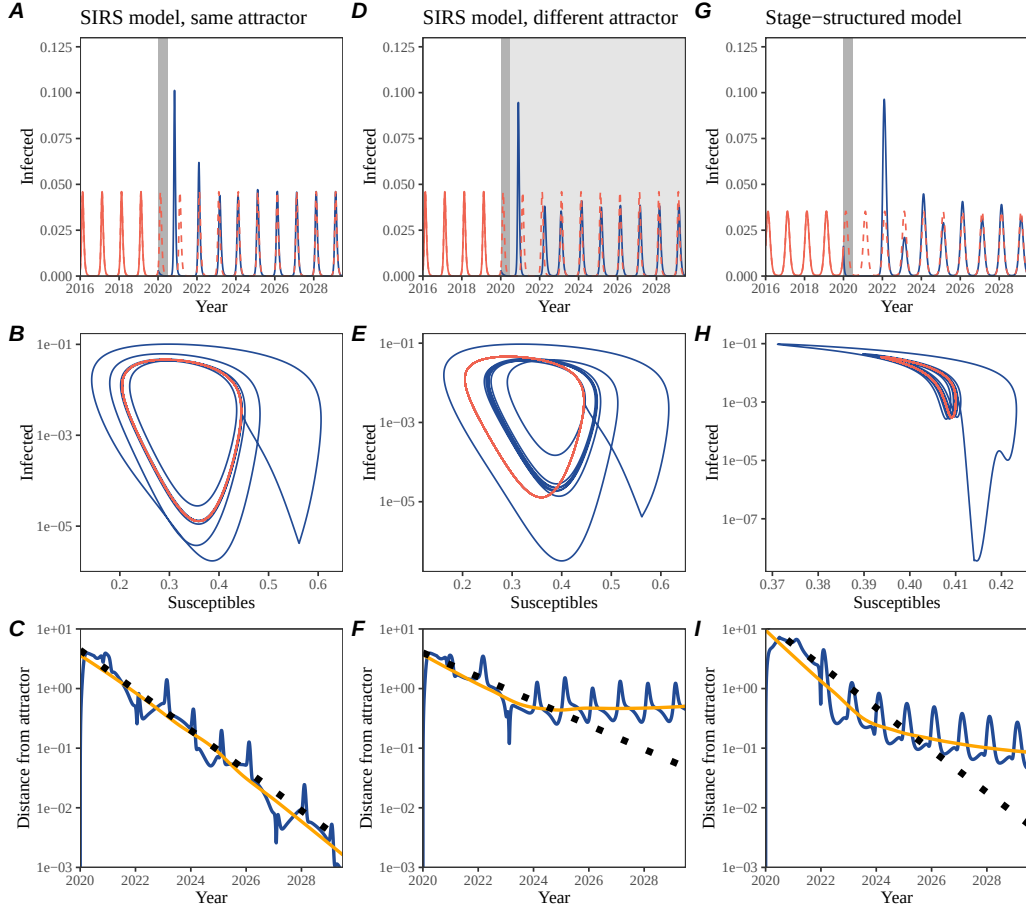
In the classical ecological literature, the resilience of an ecological system is measured by the rate at which the system is expected to return to its reference state following a perturbation [13, 14, 15, 16]. This rate corresponds to the largest real part of the eigenvalues of the linearized system near equilibrium—here, we refer to this value as the *intrinsic* resilience of the system. In practice, we rarely know the true, underlying data-generating process that describes the dynamics of common respiratory pathogens, which can depend on hidden variables, such as changes in population-level susceptibility. Instead, we have to rely on simplifying model assumptions, raising questions about whether we can infer the intrinsic resilience of a system accurately. Nonetheless, we can measure the *empirical* resilience of a host-pathogen system by looking at how fast the system returns to the pre-perturbation endemic dynamics after the perturbation has ended. The COVID-19 pandemic provides a convenient example of a major perturbation.

### **Resilience of a single-strain system under a short-term perturbation.**

As an example, we begin with a simple Susceptible-Infected-Recovered-Susceptible (SIRS) model with seasonally forced transmission and demography (i.e., birth and death) [18]. Here, a pandemic perturbation that reduces transmission by 50% for 6 months starting in 2020 causes the epidemic to deviate from its original stable annual cycle and eventually return (Figure 2A). To quantify resilience, we measure the distance from the pre-pandemic attractor—a set of points in state space or phase plane that the system is pulled towards [19]. Specifically, we measure the Euclidean distance on the SI phase plane relative to the counterfactual unperturbed trajectory (Figure 2B; Materials and Methods). This distance decreases exponentially with time on average, as demonstrated by the locally estimated scatterplot smoothing (LOESS) fit, and the overall rate of return matches the analytically derived intrinsic resilience of the seasonally unforced system (Figure 2C).

### **Resilience of a single-strain system under a long-term perturbation.**

Alternatively, pandemic perturbations can have a lasting impact on pathogen dynamics through a reduction in transmission or change in immunity. For example, if a 10% reduction in transmission persists even after the major pandemic perturbations are lifted, the system will converge to a new attractor (Figure 2D–F), but we may not be able to confirm the convergence to a new attractor until many years have passed. Nonetheless, we can still measure the distance from the pre-pandemic attractor (Figure 2E). The distance initially decreases exponentially on average (equivalently, linearly on a log scale) and eventually plateaus (Figure 2F). Here, a permanent 10% reduction in transmission rate slows the system, causing a slower decrease in the distance from the attractor (Figure 2F) than it would have otherwise (Figure 2C). This example shows that resilience is not necessarily an intrinsic property of a specific pathogen. Instead, pathogen resilience is a property of a specific attractor that a host-pathogen system approaches, which depends on both pathogen and host characteristics.



**Figure 2: A simple method to measure pathogen resilience following pandemic perturbations across different scenarios.** (A, D, G) Simulated epidemic trajectories across various models. Red and blue solid lines represent epidemic dynamics before and after pandemic perturbations are introduced, respectively. Red dashed lines represent counterfactual epidemic dynamics in the absence of perturbations. Gray regions indicate the duration of perturbations with the dark gray region representing a 50% transmission reduction and the light gray region representing a 10% transmission reduction. (B, E, H) Phase plane representation of the time series in panels A, D, and G alongside the corresponding susceptible host dynamics. Red and blue solid lines represent epidemic trajectories on an SI phase plane before and after perturbations are introduced, respectively. (C, F, I) Changes in distance from the attractor over time on a log scale. Blue lines represent the distance from the attractor. Orange lines represent the locally estimated scatterplot smoothing (LOESS) fits to the logged distance from the attractor. Dotted lines show the intrinsic resilience of the seasonally unforced system.

**Resilience of a single-strain system with long-term transients.** Finally, transient phenomena can further complicate the picture (Figure 2G–I). For example, a stage-structured model that includes a reduction in secondary susceptibility initially exhibits a stable annual cycle, but perturbations from a 50% reduction in transmission for 6 months cause the epidemic to shift to a transient biennial cycle before it eventually approaches the original pre-pandemic attractor (Figure 2G,H). As indicated by the LOESS fit, the distance from the attractor initially decreases exponentially at a rate that is consistent with the intrinsic resilience of the seasonally unforced stage-structured system, but the rate of return slows with the damped oscillations (Figure 2I). This behavior is also referred to as a ghost attractor, which causes long transient dynamics and slow transitions [20]. Strong seasonal forcing in transmission can also lead to transient phenomena for a simple SIRS model, causing a slow return to pre-perturbation dynamics (Supplementary Figure S1). However, adding a small rate of imported infections to this strongly seasonal model removes these transient dynamics and increases the system’s resilience (Supplementary Figure S2). Such a change to a non-seasonal model has considerably reduced impact on resilience (Supplementary Figure S3).

**Resilience of a two-strain system.** This empirical approach allows us to measure the resilience of a two-strain host-pathogen system even when we imperfectly observe infection dynamics. Simulations of two competing strains illustrate that separate analyses of individual strain dynamics (e.g., RSV subtype A vs B) and a joint analysis of total infections (e.g., total RSV infections) yield identical resilience estimates (Supplementary Figure S4, 5). This is expected because eigenvalues determine the dynamics of the entire system around the equilibrium, meaning that both strains should exhibit identical rates of return following a perturbation. Analogous to a single-strain system, strong seasonal forcing in transmission can cause the two-strain system to slow down through transient phenomena (Supplementary Figure S6).

These observations yield three insights. First, we can directly estimate the empirical resilience of a host-pathogen system by measuring the rate at which the system approaches an attractor. The empirical approach to estimating pathogen resilience is particularly convenient because it does not require us to know the true, underlying data-generating process; estimating the intrinsic resilience from fitting misspecified models can lead to biased estimates (Supplementary Figure S7). Second, resilience estimates allow us to make phenomenological predictions about the dynamics of a host-pathogen system following a perturbation. Assuming that an attractor has not changed and the distance from the attractor will decrease exponentially over time, we can estimate when the system should reach an attractor. Finally, a change in the (exponential) rate of approach can provide information about whether the system has reached an alternative attractor, or a ghost attractor, that is different from the original, pre-pandemic attractor. These alternative attractors may reflect permanent changes in transmission patterns as well as changes in immune landscapes. Now that several years have passed since major interventions have been lifted, many respiratory pathogens may have had sufficient time to return to their post-intervention

179 attractors—empirical comparisons between current cases and pre-pandemic cases are  
 180 consistent with this hypothesis (Figure 1). With recent data, we can start to evaluate  
 181 whether we see early signs of convergence to the former attractor or a new one.

## 182 Inferring pathogen resilience from real data

183 Based on these patterns, we now present our approach to estimating pathogen re-  
 184 siliance from real data (Figure 3). We first tested this approach against simulations  
 185 and then analyzed case time series of respiratory pathogens from Canada, Hong  
 186 Kong, Korea, and the US.

187 So far, we have focused on simple examples that assume a constant transmis-  
 188 sion reduction during the pandemic. However, in practice, the impact of pandemic  
 189 perturbations on pathogen transmission is likely more complex (Figure 3A), reflect-  
 190 ing the introduction and relaxation of various intervention strategies. For example,  
 191 continued transmission reduction from interventions can limit the size of the first  
 192 outbreak following the rebound, allowing for a larger outbreak when interventions  
 193 are further relaxed (Figure 3B).

194 Previously, we relied on the dynamics of susceptible and infected hosts to com-  
 195 pute the distance from the attractor (Figure 2), but information on susceptible hosts  
 196 is rarely available in practice. In addition, uncertainties in case counts due to obser-  
 197 vation error, strain evolution, and multiannual cycles in the observed epidemic dy-  
 198 namics (e.g., adenovirus in Hong Kong and Korea) complicate defining pre-pandemic  
 199 attractors. To address these challenges, we can reconstruct an empirical attractor by  
 200 utilizing Takens’ theorem [17], which states that an attractor of a nonlinear multidim-  
 201 ensional system can be mapped onto a delayed embedding (Materials and Meth-  
 202 ods). Specifically, by taking the logged values of pre-pandemic cases  $\log(C(t) + 1)$   
 203 and creating delayed copies, Takens’ theorem allows us to define the pre-pandemic  
 204 attractor as a set of points in an  $M$ -dimensional space:

$$\langle \log(C(t) + 1), \log(C(t - \tau) + 1), \dots, \log(C(t - (M - 1)\tau) + 1) \rangle, \quad (1)$$

205 where the delay  $\tau$  and embedding dimension  $M$  are determined based on autocorre-  
 206 lations and false nearest neighbors, respectively [21, 22]. Intuitively, Takens’ theorem  
 207 tells us that, delayed copies of the observed time series preserve the dynamical prop-  
 208 erties of the underlying multi-dimensional attractor. We can then apply the same  
 209 delay and embedding dimensions to the entire time series to determine its position in  
 210 multi-dimensional state space (Figure 3D) and measure the nearest neighbor distance  
 211 between the current state of the system and the empirical pre-pandemic attractor  
 212 (Figure 3E). In theory, the rate at which the distance from this attractor decreases,  
 213 which can be estimated by fitting a linear regression on a log scale, provides an em-  
 214 pirical measure of pathogen resilience (Figure 3E). However, resulting estimates of  
 215 pathogen resilience can be sensitive to choices about embedding delays and dimen-  
 216 sions. Longer delays and higher dimensions tend to smooth out temporal variations  
 217 in the distance from the attractor (Supplementary Figure S8).

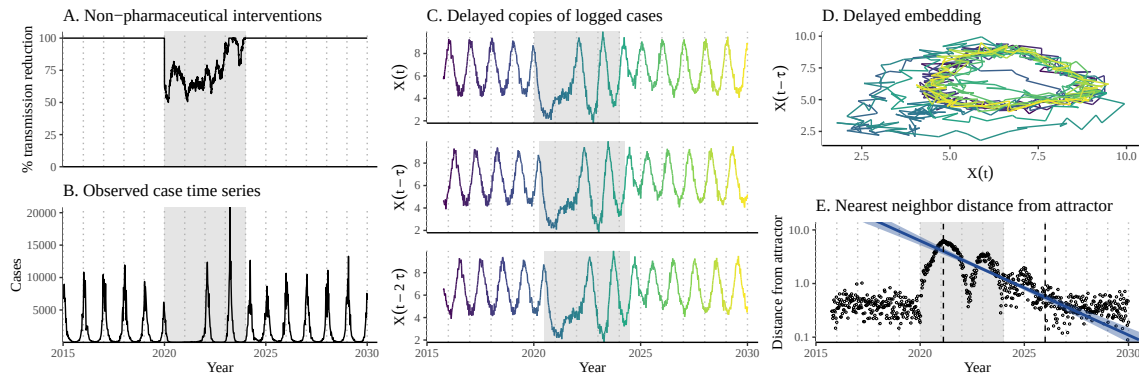


Figure 3: **A schematic diagram explaining the inference of pathogen resilience from synthetic data.** (A) A realistic example of simulated pandemic perturbations, represented by a relative reduction in transmission. (B) The impact of the synthetic pandemic perturbation on epidemic dynamics simulated using a SIRS model with demographic stochasticity. (C) Generating delayed copies of the logged time series allows us to obtain an embedding. (D) Two dimensional representation of an embedding. (E) Delayed embedding allows us to calculate the nearest neighbor distance from the empirical attractor, which is determined based on the pre-pandemic time series. Blue lines and dashed regions represent the linear regression fit and the associated 95% confidence interval. This distance time series can be used to infer pathogen resilience after choosing an appropriate window for linear regression. For illustration purposes, an arbitrary fitting window was chosen.

218 Estimating pathogen resilience from linear regression is sensitive to choices about  
 219 the windows used to fit the regression (Figure 3E). Therefore, we developed an  
 220 automated window selection criterion for linear regression and validated our crite-  
 221 rion, alongside embedding dimensions choices, against simulations (Materials and  
 222 Methods; Supplementary Results; Supplementary Figure S9). We then applied the  
 223 resulting criterion and embedding dimension to pathogen surveillance data presented  
 224 in Figure 1 (Materials and Methods; Supplementary Results; Supplementary Figure  
 225 S10-S11).

226 For all pathogens we considered, resilience estimates fall between 0.4/year and  
 227 1.8/year (Figure 4A). We estimated the mean resilience of common respiratory  
 228 pathogens to be 0.99/year (95% CI: 0.81/year–1.18/year). For reference, this is  
 229  $\approx 7.5$  times higher than the intrinsic resilience of pre-vaccination measles in England  
 230 and Wales ( $\approx 0.13$ /year). Finally, resilience estimates for norovirus, a gastrointesti-  
 231 nal pathogen, were comparable to those of common respiratory pathogens: 1.44/year  
 232 (95% CI: 1.01/year–1.87/year) for Hong Kong and 1.07/year (95% CI: 0.86/year–  
 233 1.29/year) for Korea. Based on a simple ANOVA test, we did not find significant  
 234 differences in resilience estimates across countries ( $p = 0.25$ ) or pathogens ( $p = 0.67$ ).

235 Using resilience estimates, we predicted when each pathogen would hypothetically  
 236 return to their pre-pandemic dynamics, assuming no long-term change in the attrac-

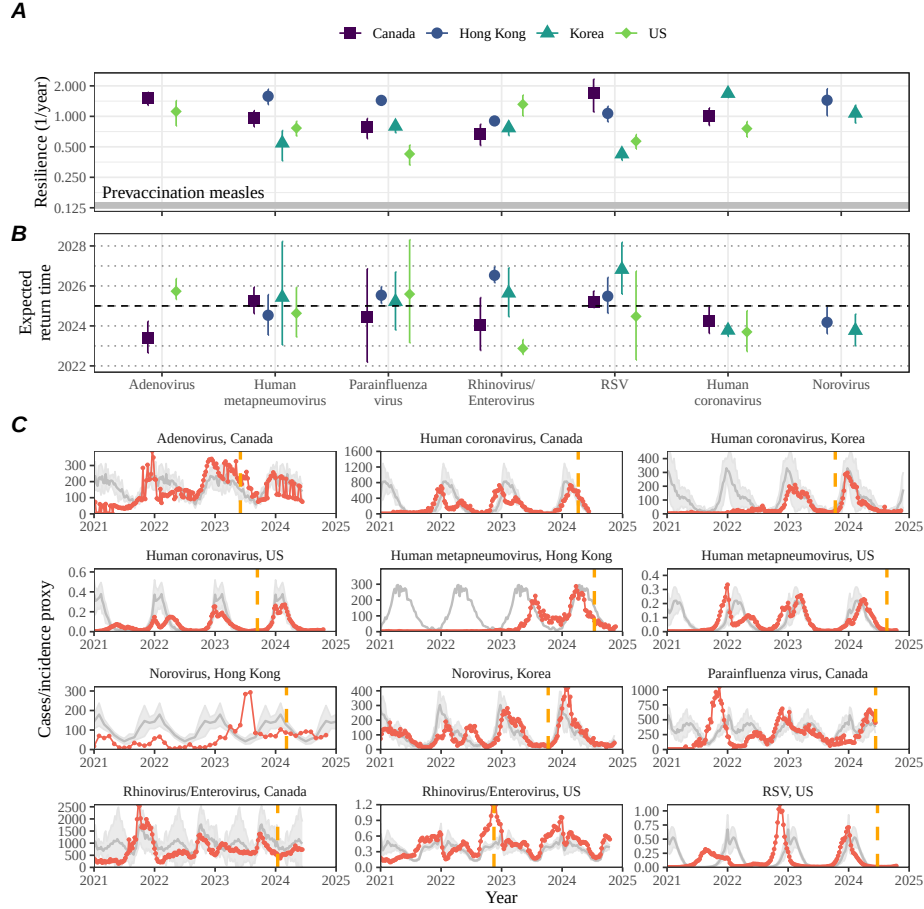


Figure 4: **Summary of resilience estimates and predictions for return time.** (A) Estimated pathogen resilience. The gray horizontal line represents the intrinsic resilience of pre-vaccination measles dynamics. (B) Predicted timing of when each pathogen will return to their pre-pandemic cycles. The dashed line in panel B indicates the end of 2024. Error bars represent 95% confidence intervals. (C) Observed dynamics for pathogen that are predicted to have returned before the end of 2024. Red points and lines represent data before 2020. Gray lines and shaded regions represent the mean seasonal patterns and corresponding 95% confidence intervals around the mean, previously shown in Figure 1. Orange vertical lines represent the predicted timing of return.

tor. Specifically, we extended our linear regression fits to distance-from-attractor time series and ask when the predicted regression line will cross a threshold value; since we relied on nearest neighbor distances, pre-pandemic distances are always greater than zero (Figure 3E), meaning that we can use the mean of pre-pandemic distances as our threshold.

We predicted that a return to pre-pandemic cycles has occurred or would be

243 imminent for most pathogens (Figure 4B). In particular, we predicted that 12 out  
 244 of 23 pathogen-country pairs should have already returned before the end of 2024.  
 245 For almost all pathogens that were predicted to have returned already, the observed  
 246 epidemic dynamics showed clear convergence towards their pre-pandemic seasonal  
 247 averages, confirming our predictions (Figure 4C). However, there were a few ex-  
 248 ceptions, including norovirus in Hong Kong and rhinovirus/enterovirus in the US,  
 249 where the observed epidemic dynamics in 2024 exhibit clear deviation from their pre-  
 250 pandemic seasonal averages (Figure 4C; Figure S11). These observations suggest a  
 251 possibility that some common respiratory pathogens may have converged to different  
 252 attractors or are still exhibiting non-equilibrium dynamics. In contrast, pathogens  
 253 that were predicted to have not returned yet also showed clear differences from their  
 254 pre-pandemic seasonal averages; as many of these pathogens are predicted to return  
 255 in 2025–2026, we may be able to test these predictions in near future (Supplementary  
 256 Figure S12). Our reconstructions of distance time series and estimates of pathogen  
 257 resilience and expected return time were generally robust to choices of embedding  
 258 dimensions (Supplementary Figure S13–14). We found that making out-of-sample  
 259 predictions were challenging, but still supported consistency in our predictions (Sup-  
 260 plementary Results; Supplementary Figure S15–16).

## 261 **Susceptible host dynamics explain variation in pathogen** 262 **resilience**

263 So far, we have focused on quantifying pathogen resilience from the observed pat-  
 264 terns of pathogen re-emergence following pandemic perturbations. But what factors  
 265 determine the resilience of a host-pathogen system? Here, we illustrate how sus-  
 266 ceptible host dynamics affect pathogen resilience by varying the basic reproduction  
 267 number  $\mathcal{R}_0$  and the duration of immunity of the SIRS model.

268 An increase in  $\mathcal{R}_0$  and a decrease in the duration of immunity increase resilience  
 269 (Figure 5A) and the per-capita rate of susceptible replenishment, the ratio between  
 270 the total rate at which new susceptibles enter the population and the equilibrium  
 271 number of susceptibles,  $S^*$  (Figure 5B). Specifically, a higher  $\mathcal{R}_0$  drives a faster per-  
 272 capita susceptible replenishment rate by decreasing  $S^*$ . For a simple SIR model that  
 273 assumes life-long immunity, pathogen resilience is proportional to the per-capita rate  
 274 of susceptible replenishment (Materials and Methods). Overall, these observations  
 275 suggest that a faster per-capita susceptible replenishment rate causes the system to  
 276 be more resilient.

277 By taking the average values of empirical resilience for each pathogen, we were  
 278 able to map each pathogen onto a set of parameters of the SIRS model that are  
 279 consistent with corresponding resilience estimates (Figure 5A). Across all pathogens  
 280 we considered, we estimated that the average duration of immunity is likely to be  
 281 short ( $< 4$  years) across a plausible range of  $\mathcal{R}_0$  ( $< 6$ ). In contrast, there was  
 282 large uncertainty in the estimates for susceptible replenishment rates due to a lack

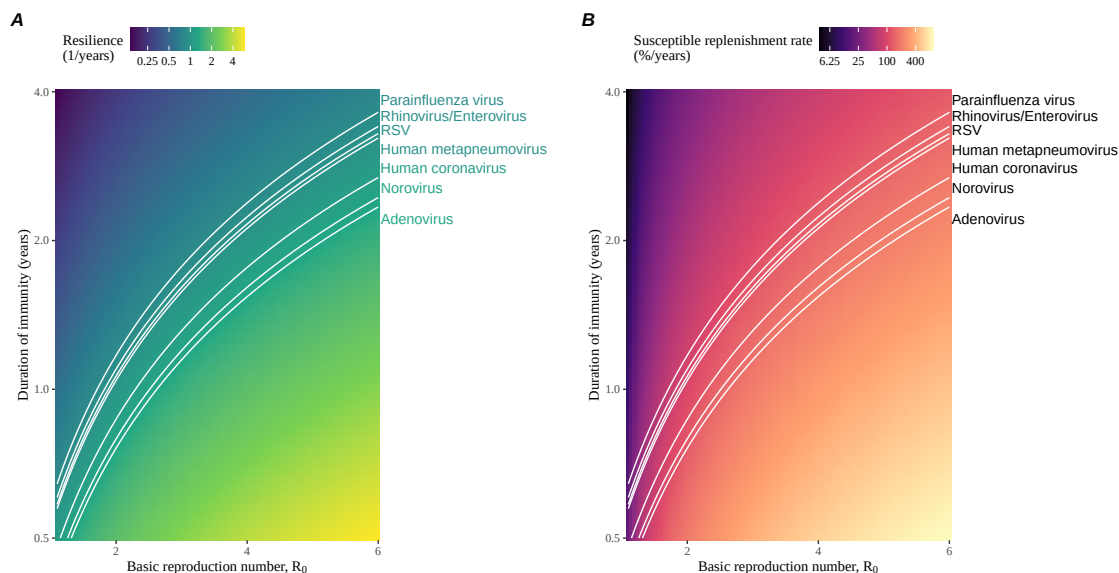


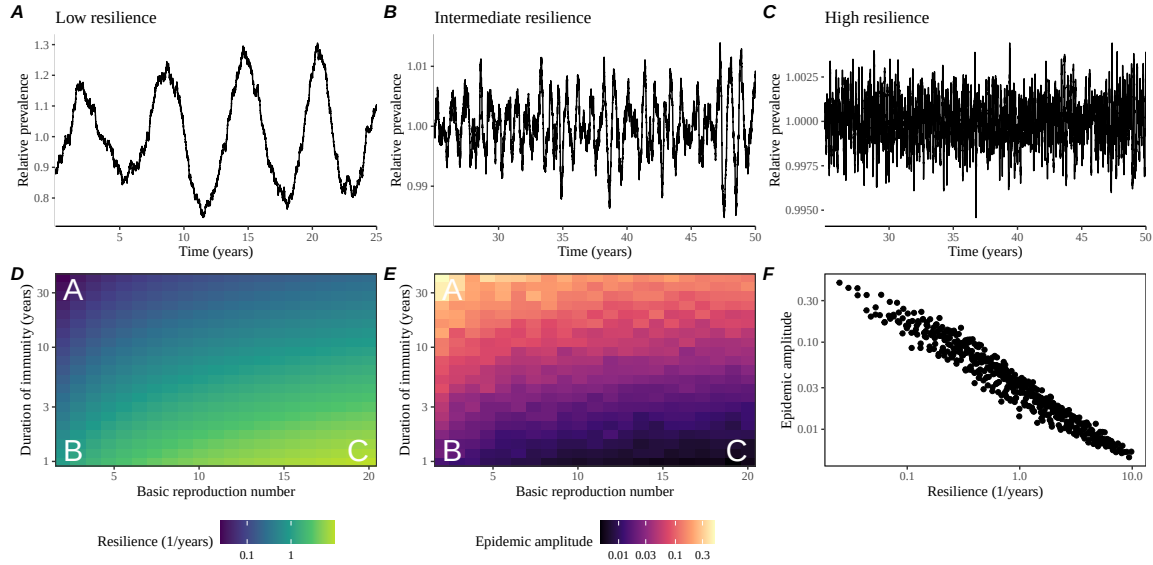
Figure 5: **Linking pathogen resilience to epidemiological parameters and susceptible host dynamics.** (A) Intrinsic resilience as a function of the basic reproduction number  $\mathcal{R}_0$  and the duration of immunity. (B) Per-capita susceptible replenishment rate as a function of the basic reproduction number  $\mathcal{R}_0$  and the duration of immunity. The standard SIRS model without seasonal forcing is used to compute intrinsic resilience and the per-capita susceptible replenishment rate. Lines correspond to a set of parameters that are consistent with mean resilience estimates for each pathogen, averaged across different countries.

283 of one-to-one correspondence between susceptible replenishment rates and pathogen  
 284 resilience (Figure 5B).

## 285 Pathogen resilience determines sensitivity to stochas- 286 tic perturbations

287 Even in the absence of major pandemics, host-pathogen systems are expected to  
 288 experience continued perturbations of varying degrees from changes in epidemiolog-  
 289 ical conditions, such as human behavior, climate, and viral evolution. These per-  
 290 turbations can also arise from demographic stochasticity, which is inherent to any  
 291 ecological system. Here, a seasonally unforced SIRS model with constant birth and  
 292 death rates illustrates that resilience of a host-pathogen system determines the sen-  
 293 sitivity to perturbations caused by demographic stochasticity (Figure 6, Materials  
 294 and Methods). Less resilient systems exhibits greater deviations from the deter-  
 295 ministic trajectory and slower epidemic cycles (Figure 6A–C). The periodicity of  
 296 this epidemic cycle matches those predicted by the intrinsic periodicity of the sys-

tem (Supplementary Figure S17), which is inversely proportional to the intrinsic  
resilience (Supplementary Figure S18).



**Figure 6: Linking resilience of a host-pathogen system to its sensitivity to stochastic perturbations.** (A–C) Epidemic trajectories of a stochastic SIRS model across three different resilience values: low, intermediate, and high. The relative prevalence was calculated by dividing infection prevalence by its mean value. (D) Intrinsic resilience of a system as a function of the basic reproduction number  $\mathcal{R}_0$  and the duration of immunity. (E) Epidemic amplitude as a function of the basic reproduction number  $\mathcal{R}_0$  and the duration of immunity. The epidemic amplitude corresponds to  $(\max I - \min I)/(2\bar{I})$ , where  $\bar{I}$  represents the mean prevalence. Labels A–C in panels D and E correspond to scenarios shown in panels A–C. (F) The relationship between pathogen resilience and epidemic amplitude.

The intrinsic resilience is not the sole determinant for of a system’s sensitivity  
to stochastic perturbations. For example, the population size and average duration  
of infection also affect the amount of deviation from the deterministic trajectory  
caused by demographic stochasticity, even though these quantities have little to no  
impact on the intrinsic resilience (Supplementary Figure S17). These conclusions  
were robust for the seasonally forced SIRS model, where the amount of deviation  
from the deterministic trajectory depends on the corresponding resilience of the  
seasonally unforced system (Supplementary Figure S19).

## Discussion

COVID-19 pandemic interventions caused major disruptions to circulation patterns of both respiratory and non-respiratory pathogens, adding challenges to predicting their future dynamics [1, 2, 3, 4]. These perturbations offer large-scale natural experiments for understanding how different pathogens respond to perturbations. In this study, we showed that pathogen re-emergence patterns following pandemic perturbations can be characterized through the lens of ecological resilience and presented a new method for estimating pathogen resilience from time series data. We showed that variation in pathogen resilience can be explained by the differences in susceptible host dynamics, where faster replenishment of the susceptible pool corresponds to a more resilient host-pathogen system. Finally, we showed that pathogen resilience also determines the sensitivity to stochastic perturbations.

We analyzed case time series of common respiratory infections and norovirus infections from Canada, Hong Kong, Korea, and the US to estimate their resilience. Overall, we estimated the resilience of these pathogens to range from 0.4/year to 1.8/year, which is 3–14 times more resilient than prevaccination measles. Consistent with other epidemiological evidence [23, 24, 25, 26], these resilience estimates indicate that common respiratory pathogens and norovirus likely exhibit faster susceptible replenishment and are therefore more persistent, indicating potential challenges in controlling these pathogens.

Based on our resilience estimates, we made phenomenological predictions about when each pathogen will return to their endemic cycles. Our main regression analysis could accurately distinguish which pathogens should have already returned before the end of 2024. However, there were two main exceptions (i.e., norovirus in Hong Kong and rhinovirus/enterovirus in the US), hinting at long-lasting impact of pandemic perturbations. These changes may reflect changes in surveillance or the dynamics. While it may seem unlikely that behavioral changes would only affect a few pathogens and not others, we cannot rule out this possibility given differences in the observed mean age of infections and therefore the differences in age groups that primarily drive transmission [27, 28]. Differences in the mode of transmission between respiratory vs gastrointestinal pathogens may also contribute to the differences in responses to pandemic perturbations.

For almost half of the pathogens we considered, we predicted that their return to original epidemic patterns is imminent. However, our evaluation of out-of-sample predictions suggested major challenges in predicting the exact timing of pathogen return and estimating pathogen resilience from limited data. Therefore, our estimates of prediction timing should be taken as a ballpark estimate. We will need a few more years of data to test whether these pathogens will eventually return to their original dynamics or eventually converge to a different attractor. We also cannot rule out the possibility that some pathogens may exhibit long-term transient behaviors following pandemic perturbations. Overall, these observations echo earlier studies that highlighted the long-lasting impact of pandemic perturbations [8, 29, 30, 4, 31].

349 We showed that susceptible host dynamics shape pathogen resilience, where faster  
350 replenishment of the susceptible population causes the pathogen to be more resilient.  
351 For simplicity, we focused on waning immunity and birth as the main drivers of the  
352 susceptible host dynamics but other mechanisms can also contribute to the replen-  
353 ishment of the susceptible population. In particular, pathogen evolution, especially  
354 the emergence of antigenically novel strains, can cause effective waning of immunity  
355 in the population; therefore, we hypothesize that the rate of antigenic evolution is  
356 likely a key feature of pathogen resilience. Future studies should explore the relation-  
357 ship between the rate of evolution and resilience for antigenically evolving pathogens.  
358 This result also highlights the importance of detailed measurements of changes in the  
359 susceptible population through immune assays for understanding pathogen dynamics  
360 [32].

361 Many other factors may affect the resilience of a host-pathogen system beyond  
362 susceptible replenishment rate. For example, we showed that importation can also  
363 increase the resilience of the system (Supplementary Figures S2, 3). Interestingly,  
364 this effect is minimal without seasonal forcing but amplified under strong seasonal  
365 forcing, suggesting that analyzing the eigenvalues of the seasonally unforced system  
366 may be insufficient to capture key aspects of the system’s resilience. Future studies  
367 should consider both analytical and numerical analyses of more detailed, mechanistic  
368 models to understand how other factors—such as the generation-interval distribution  
369 [33], immune boosting [34], strain competition [35], and post-infection mortality  
370 [36]—may alter resilience. Quantifying relative changes in pathogen resilience with  
371 respect to underlying parameters of a model—analogue to sensitivity and elasticity  
372 analyses in ecology [37, 38]—may allow one to infer relative importance of different  
373 factors that affect pathogen resilience.

374 There are several limitations to our work. First, we did not extensively explore  
375 other approaches to reconstructing the attractor. Recent studies showed that more  
376 sophisticated approaches, such as using non-uniform embedding, can provide more  
377 robust reconstruction for noisy data [22]. In the context of causal inference, choices  
378 about embedding can have big impacts on the resulting inference [39]. Our resilience  
379 estimates are likely overly confident given a lack of uncertainties in attractor re-  
380 construction as well as the simplicity of our statistical framework. Nonetheless, as  
381 illustrated in our sensitivity analyses, inferences about pathogen resilience in our  
382 SIRS model appear to be robust to decisions about embedding lags and dimensions.  
383 Short pre-pandemic time series also limit our ability to accurately reconstruct the  
384 attractor and contribute to the crudeness of our resilience estimates. Our framework  
385 also does not allow us to distinguish whether a system has settled to a new attractor  
386 or is experiencing long-term transient behavior.

387 Uncertainties in pathogen dynamics due to changes in testing patterns further  
388 contribute to the crudeness of our resilience estimates. We tried to account for  
389 changes in testing patterns by either scaling the case counts (for Canada, Hong  
390 Kong, and Korea) or deriving an incidence proxy (for the US). We were unable to  
391 derive an incidence proxy for Canada and Hong Kong due to instability of the test

positivity ratio. Therefore, we cannot rule out the possibility the apparent long-term impact of pandemic perturbations may simply reflect changes in testing and reporting patterns, rather than actual changes in the dynamics. More stable proxies of incidence (e.g., from wastewater) might be useful in future studies.

Finally, our simulation-based analyses primarily focused on single-strain systems, but real-world pathogens can interact with other pathogens, which can result in complex dynamics [40, 41]. To address this limitation, we considered a simple model of two competing strains (via cross immunity) and showed that the resilience of a coupled system can be measured by studying the dynamics of either strain. However, this conclusion likely depends on the strength and mechanism of strain interactions. For example, ecological interference between two unrelated pathogens [40] will likely generate weaker coupling than cross-immunity between related pathogens; in the former case, we do not necessarily expect two unrelated pathogens to have the same resilience despite their ecological interference. Some pathogen strains can also exhibit positive interactions where infection by one strain can lead to an increased transmission of another competing strain. Our study nonetheless illustrates that quantifying pathogen resilience can provide novel insights into pathogen dynamics. Furthermore, our qualitative prediction that common respiratory pathogens are more resilient than prevaccination measles is also likely to be robust, given how rapidly many respiratory pathogens returned to their original cycles following pandemic perturbations.

Predicting the impact of anthropogenic changes on infectious disease dynamics is a fundamental aim. Our study illustrates that how a host-pathogen system responds to both small and large perturbations is largely predictable through the lens of ecological resilience. In particular, quantifying the resilience of a host-pathogen system offers insight into endemic pathogens' responses to pandemic perturbations, including whether some pathogens will exhibit long-lasting impact from the pandemic perturbation or not. More broadly, a detailed understanding of the determinants of pathogen resilience might improve understanding of pathogen persistence.

## Materials and Methods

### Data

We gathered time series on respiratory infections from Canada, Hong Kong, Korea, and United States (US). As a reference, we also included norovirus infections when available. In contrast to respiratory pathogens, we hypothesized gastrointestinal viruses, such as norovirus, to be differently affected by pandemic perturbations.

Weekly time series of respiratory infection cases in Canada came from a publicly available website by the Respiratory Virus Detection Surveillance System, which collects data from select laboratories across Canada [42]. Weekly time series of respiratory infection cases in Hong Kong came from a publicly available website by the Centre for Health Protection, Department of Health [43, 44]. Weekly time series of acute respiratory infection cases in Korea came from a publicly available

432 website by the Korea Disease Control and Prevention Agency [45]. Finally, weekly  
433 time series of respiratory infection cases in the US were obtained from the National  
434 Respiratory and Enteric Virus Surveillance System (NREVSS). Readers can request  
435 the data from NREVSS at nrevss@cdc.gov. Time series on number of tests were also  
436 available in Canada, Hong Kong, and the US, but not in Korea.

## 437 Data processing

438 For all time series, we rounded every year to 52 weeks by taking the average number  
439 of cases and tests between the 52nd and 53rd week. We also rescaled all time series to  
440 account for changes in testing patterns, which were then used for the actual analysis.

441 For Canada, an increase in testing was observed from 2013 to 2024 (Supplemen-  
442 tary Figure S21); this increase is associated with a decrease in test positivity during  
443 the same period (Supplementary Figure S22). To account for this increase, we cal-  
444 culated a 2 year moving average for the number of tests for each pathogen, which we  
445 used as a proxy for testing effort. Then, we divided the smoothed testing patterns  
446 by the smoothed value at the final week such that the testing effort has a maximum  
447 of 1. We then divided weekly cases by the testing effort to obtain a scaled case time  
448 series. A similar approach was used earlier for an analysis of RSV time series in the  
449 US to account for changes in testing patterns [25].

450 For Hong Kong, we applied the same scaling procedure to the time series as  
451 we did for Canada. In this case, we only adjusted for testing efforts up to the  
452 end of 2019 because there was a major reduction in testing for common respiratory  
453 pathogens between 2020 and 2023 (Supplementary Figure S23). Analogous to pat-  
454 terns in Canada, an increase in testing volume is associated with a decrease in test  
455 positivity (Supplementary Figure S24).

456 For Korea, while we did not have information on testing, the reported number  
457 of respiratory infections consistently increased from 2013 to the end of 2019, which  
458 we interpreted as changes in testing patterns (Supplementary Figure S25). Since  
459 we did not have testing numbers, we used the weekly sum of all acute respiratory  
460 viral infection cases as a proxy for testing, which were further smoothed with moving  
461 average and scaled to have a maximum of 1. For Korea, we also only adjusted for  
462 testing efforts up to the end of 2019.

463 In the US, there has been a large increase in testing for some respiratory pathogens,  
464 especially RSV, which could not be corrected by simple scaling (Supplementary Fig-  
465 ure S26). Instead, we derived an incidence proxy by multiplying the test positivity  
466 with influenza-like illness positivity, which was taken from [https://gis.cdc.gov/  
467 grasp/fluview/fluportaldashboard.html](https://gis.cdc.gov/grasp/fluview/fluportaldashboard.html). This method of estimating an inci-  
468 dence proxy has been recently applied in the analysis of seasonal coronaviruses [7]  
469 and *Mycoplasma pneumoniae* infections [4]. Detailed assumptions and justifications  
470 are provided in [46].

## 471 Data summary

472 To make qualitative comparisons between pre- and post-perturbation dynamics of  
 473 respiratory pathogen circulation patterns, we calculated the mean seasonal patterns  
 474 using time series of either rescaled cases or incidence proxy estimates before 2020. We  
 475 did so by taking the mean value in each week across all years before 2020. Confidence  
 476 intervals around the means were calculated using a simple t test.

## 477 Estimating pathogen resilience

478 In order to measure pathogen resilience from surveillance data, we first reconstructed  
 479 the empirical pre-pandemic attractor of the system using Takens' embedding theorem  
 480 [17]. Specifically, for a given pathogen, we took the pre-pandemic (before 2020) case  
 481 time series  $C(t)$  and reconstructed the attractor using delayed embedding with a  
 482 uniform delay of  $\tau$  and dimension  $M$ :

$$X_{\tau,m}(t) = \langle \log(C(t) + 1), \log(C(t - \tau) + 1), \dots, \log(C(t - (M - 1)\tau) + 1) \rangle. \quad (2)$$

483 Here, the delay  $\tau$  was determined by calculating the autocorrelation of the logged  
 484 pre-pandemic time series and asking when the autocorrelation crosses 0 for the first  
 485 time [22]; a typical delay for an annual outbreak is around 13 weeks.

486 Then, for a given delay  $\tau$ , we determined the embedding dimension  $M$  using the  
 487 false nearest neighbors approach [21, 22]. To do so, we started with an embedding  
 488 dimension  $e$  and constructed a set of points  $A_{\tau,e} = \{X_{\tau,e}(t) | t < 2020\}$ . Then, for  
 489 each point  $X_{\tau,e}(t)$ , we determined the nearest neighbor from the set  $A_{\tau,e}$ , which we  
 490 denote  $X_{\tau,e}(t_{nn})$  for  $t \neq t_{nn}$ . Then, if the distance between these two points in the  
 491  $e+1$  dimension,  $D_{\tau,e+1}(t) = \|X_{\tau,e+1}(t_{nn}) - X_{\tau,e+1}(t)\|_2$ , is larger than their distance in  
 492 the  $e$  dimension,  $D_{\tau,e}(t) = \|X_{\tau,e}(t_{nn}) - X_{\tau,e}(t)\|_2$ , these two points are deemed to be  
 493 false nearest neighbors; specifically, we used a threshold  $R$  for the ratio between two  
 494 distances  $D_{\tau,e+1}(t)/D_{\tau,e}(t)$  to determine false nearest neighbors. The first embed-  
 495 ding dimension  $e$  that does not have any false nearest neighbors corresponds to the  
 496 embedding dimension  $M$  for a given pathogen-country pair. For the main analysis,  
 497 we used  $R = 10$ , which was chosen from a sensitivity analysis against simulated data  
 498 (Supplementary Text). Once we determined the embedding lag  $\tau$  and dimension  $M$ ,  
 499 we apply the embedding to the entire time series and calculate the nearest neigh-  
 500 bor distance against the attractor  $A_{\tau,M}$  to obtain a time series of distance from the  
 501 attractor  $D_{\tau,M}(t)$ .

502 From a time series of distances from the attractor, we estimated pathogen re-  
 503 siliance by fitting a linear regression to an appropriate window. To automatically  
 504 select fitting windows, we began by smoothing the distance time series using locally  
 505 estimated scatterplot smoothing (LOESS) to obtain  $\hat{D}_{\tau,M}(t)$ , where the smoothing  
 506 is performed on a log scale and exponentiated afterwards. This smoothing allowed  
 507 us to find appropriate threshold values for selecting fitting windows that are insensi-  
 508 tive to errors in our estimates of distance from the attractor. Then, we determined

threshold values ( $T_{\text{start}}$  and  $T_{\text{end}}$ ) for the smoothed distances and choose the fitting window based on when  $\hat{D}_{\tau,M}(t)$  crosses these threshold values for the first time. These thresholds were determined by first calculating the maximum distance,

$$\max \hat{D} = \max \hat{D}_{\tau,M}(t), \quad (3)$$

and the mean pre-pandemic distance,

$$\hat{D}_{\text{mean}} = \frac{1}{N_{t < 2020}} \sum_{t < 2020} \hat{D}_{\tau,M}(t), \quad (4)$$

as a reference, and then dividing their ratios into  $K$  equal bins,

$$T_{\text{start}} = \hat{D}_{\text{mean}} \times \left( \frac{\max \hat{D}}{\hat{D}_{\text{mean}}} \right)^{(K-a)/K}, \quad (5)$$

$$T_{\text{end}} = \hat{D}_{\text{mean}} \times \left( \frac{\max \hat{D}}{\hat{D}_{\text{mean}}} \right)^{a/K}, \quad (6)$$

where  $a$  represents the truncation threshold. This allows us to discard the initial period during which the distance increases (from the introduction of intervention measures) and the final period during which the distance plateaus (as the system reaches an attractor). The fitting window is determined based on when the smoothed distance  $\hat{D}_{\tau,M}(t)$  crosses these threshold values for the first time; then, we fit a linear regression to logged (unsmoothed) distances  $\log D_{\tau,M}(t)$  using that window. Alongside the threshold  $R$  for the false nearest neighbors approach, we tested optimal choices for  $K$  and  $a$  values using simulations (Supplementary Text). We used  $K = 19$  and  $a = 2$  throughout the paper based on the simulation results. We excluded data sets from the regression analysis if the logged average of the estimated distance from the attractor during the last year is at least 2-fold greater than the logged average of the pre-pandemic distance from the attractor.

To evaluate the ability to predict the precise return time for out-of-sample observations, we applied the same framework using data up to the 26th week of 2023. In doing so, we compared the predicted return time with the observed return time, which was determined based on when the smoothed empirical distance from the attractor became close enough to pre-pandemic levels. As a pathogen returns to its pre-pandemic cycle, the the smoothed empirical distance from the attractor is expected to converge to the pre-pandemic levels  $\hat{D}_{\text{mean}}$ , but the empirical distance may not become smaller than  $\hat{D}_{\text{mean}}$ . Therefore, we used  $1.2\hat{D}_{\text{mean}}$  as our threshold to obtain an approximate return time. We note that this approach necessarily underestimates the observed return time and therefore should be interpreted as a conservative estimate. For additional validation, we compared these out-of-sample predictions with return time predictions based on full time series.

## 538 Mathematical modeling

539 Throughout the paper, we use a series of mathematical models to illustrate the  
 540 concept of pathogen resilience and to understand the determinants of pathogen re-  
 541 siliance. In general, the intrinsic resilience of a given system is given by the largest  
 542 real part of the eigenvalues of the linearized system at endemic equilibrium. Here, we  
 543 focus on the SIRS model with demography (birth and death) and present the details  
 544 of other models in Supplementary Text. The SIRS (Susceptible-Infected-Recovered-  
 545 Susceptible) model is the simplest model that allows for waning of immunity, where  
 546 recovered (immune) individuals are assumed to become fully susceptible after an  
 547 average of  $1/\delta$  time period. The dynamics of the SIRS model is described by the  
 548 following set of differential equations:

$$\frac{dS}{dt} = \mu - \beta(t)SI - \mu S + \delta R \quad (7)$$

$$\frac{dI}{dt} = \beta(t)SI - (\gamma + \mu)I \quad (8)$$

$$\frac{dR}{dt} = \gamma I - (\delta + \mu)R \quad (9)$$

549 where  $\mu$  represents the birth and death rates,  $\beta(t)$  represents the time-varying trans-  
 550 mission rate, and  $\gamma$  represents the recovery rate. The basic reproduction number  
 551  $\mathcal{R}_0(t) = \beta(t)/(\gamma + \mu)$  is defined as the average number of secondary infections that  
 552 a single infected individual would cause in a fully susceptible population at time  $t$   
 553 and measures the intrinsic transmissibility of a pathogen.

554 When we introduced the idea of pathogen resilience (Figure 2), we imposed sinu-  
 555 soidal changes to the transmission rate to account for seasonal transmission,

$$\beta(t) = b_1(1 + \theta \cos(2\pi(t - \phi)))\alpha(t), \quad (10)$$

556 where  $b_1$  represents the baseline transmission rate,  $\theta$  represents the seasonal ampli-  
 557 tude, and  $\phi$  represents the seasonal offset term. Here, we also introduced an extra  
 558 multiplicative term  $\alpha(t)$  to account for the impact of pandemic perturbations, where  
 559  $\alpha(t) < 1$  indicates transmission reduction. Figure 2A and 2B were generated assum-  
 560 ing  $b_1 = 3 \times (365/7 + 1/50)/\text{years}$ ,  $\theta = 0.2$ ,  $\phi = 0$ ,  $\mu = 1/50/\text{years}$ ,  $\gamma = 365/7/\text{years}$ ,  
 561 and  $\delta = 1/2/\text{years}$ . Specifically,  $b_1 = 3 \times (365/7 + 1/50)/\text{years}$  implies  $\mathcal{R}_0 = 3$ , where  
 562  $(365/7 + 1/50)/\text{years}$  represent the rate of recovery. In Figure 2A, we imposed a 50%  
 563 transmission reduction for 6 months from 2020:

$$\alpha(t) = \begin{cases} 0.5 & 2020 \leq t < 2020.5 \\ 1 & \text{otherwise} \end{cases} \quad (11)$$

564 In Figure 2B, we imposed a 50% transmission reduction for 6 months from 2020 and  
 565 a permanent 10% reduction onward:

$$\alpha(t) = \begin{cases} 1 & t < 2020 \\ 0.5 & 2020 \leq t < 2020.5 \\ 0.9 & 2020.5 \leq t \end{cases} \quad (12)$$

566 In both scenarios, we simulated the SIRS model from the same initial conditions  
 567 ( $S(0) = 1/\mathcal{R}_0$ ,  $I(0) = 1 \times 10^{-6}$ , and  $R(0) = 1 - S(0) - I(0)$ ) from 1900 until 2030.  
 568 When we include importations (Supplementary Figures S2 and S3), we model the  
 569 force of infection as  $\beta(t)(I + \omega)$  instead and vary  $\omega$  between  $10^{-7}$  and  $10^{-4}$  per day.  
 570 Throughout the paper, all deterministic models were solved using the `lsoda` solver  
 571 from the `deSolve` package [47] in R [48].

572 To measure the empirical resilience of the SIRS model (Figure 2C and 2F), we  
 573 computed the normalized distance between post-intervention susceptible and logged  
 574 infected proportions and their corresponding unperturbed values at the same time:

$$\sqrt{\left(\frac{S(t) - S_{\text{unperturbed}}(t)}{\sigma_S}\right)^2 + \left(\frac{\log I(t) - \log I_{\text{unperturbed}}(t)}{\sigma_{\log I}}\right)^2}, \quad (13)$$

575 where  $\sigma_S$  and  $\sigma_{\log I}$  represent the standard deviation in the unperturbed susceptible  
 576 and logged infected proportions. The unperturbed values were obtained by simulat-  
 577 ing the same SIRS model without pandemic perturbations ( $\alpha = 1$ ). We normalized  
 578 the differences in susceptible and logged infected proportions to allow both quantities  
 579 to equally contribute to the changes in distance from the attractor. We used logged  
 580 prevalence, instead of absolute prevalence, in order to capture epidemic dynamics  
 581 in deep troughs during the intervention period. In Supplementary Materials, we  
 582 also compared how the degree of seasonal transmission affects empirical resilience  
 583 by varying  $\theta$  from 0 to 0.4; when we assumed no seasonality ( $\theta = 0$ ), we did not  
 584 normalize the distance because the standard deviation of pre-intervention dynamics  
 585 are zero.

586 We used the SIRS model to understand how underlying epidemiological param-  
 587 eters affect pathogen resilience and determine the relationship to underlying sus-  
 588 ceptible host dynamics. For the simple SIRS model without seasonal transmission  
 589 ( $\theta = 0$ ), the intrinsic resilience equals

$$-\text{Re}(\lambda) = \frac{\delta + \beta I^* + \mu}{2}. \quad (14)$$

590 Here,  $I^*$  represents the prevalence at endemic equilibrium:

$$I^* = \frac{(\delta + \mu)(\beta - (\gamma + \mu))}{\beta(\delta + \gamma + \mu)}. \quad (15)$$

591 The susceptible replenishment rate is given by

$$r_S = \frac{\mu + \delta R}{S^*}, \quad (16)$$

592 where  $S^* = 1/\mathcal{R}_0$  represents the equilibrium proportion of susceptible individuals.  
 593 We varied the basic reproduction number  $\mathcal{R}_0$  between 1.1 to 6 and the average  
 594 duration of immunity  $1/\delta$  between 2 to 4 years, and computed these two quantities. In  
 595 doing so, we fixed all other parameters:  $\mu = 1/80/\text{years}$  and  $\gamma = 365/7/\text{years}$ . When

infection provides a life-long immunity ( $\delta = 0$ ), the model collapses to the SIR model and the intrinsic resilience is exactly proportional to the susceptible replenishment rate:

$$-\text{Re}(\lambda) = \frac{\mu}{2S^*} = \frac{r_S}{2}. \quad (17)$$

Finally, we used a seasonally unforced stochastic SIRS model without demography to understand how pathogen resilience affects sensitivity of the system to demographic stochasticity (see Supplementary Text for the details of the stochastic SIRS model). By varying the basic reproduction number  $\mathcal{R}_0$  between 2 to 20 and the average duration of immunity  $1/\delta$  between 1 to 40 years, we ran the SIRS model for 100 years and computed the epidemic amplitude, which we defined as  $(\max I - \min I)/(2\bar{I})$ . Each simulation began from the equilibrium, and we truncated the initial 25 years before computing the epidemic amplitude. In doing so, we assumed  $\gamma = 365/7/\text{years}$  and fixed the population size to 1 billion to prevent any fadeouts. We also considered a seasonally forced stochastic SIRS model without demography, assuming an amplitude of seasonal forcing of 0.04; in this case, we computed the relative epidemic amplitude by comparing the deterministic and stochastic trajectories (Supplementary Materials).

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## Data availability

All data and code are stored in a publicly available GitHub repository (<https://github.com/cobeylab/return-time>).

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